

The content shoot

Eight hours. Three sites, five settings. Around fifty publishable assets, localised to Lagos.

PREPARED FOR

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BY

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STATUS

Production brief. Scripts to be signed off before the day.



What this shoot is for, and what it must not repeat

She already has three hundred and three videos. They are good, they are in her own voice, and they establish the thesis: that ageing is not a problem to be solved, that the best work is the work nobody can point at, that knowing when not to operate matters more than the menu. Nothing in this shoot should re-argue any of that.

What that library cannot do is convert a Lagos buyer, for one reason: it was made for an American audience and it never mentions Nigeria. The woman in Ikoyi weighing this up is not asking what a tummy tuck is. She is asking who will be holding the instrument, what happens if something goes wrong at two in the morning, whether she will look obvious, and whether she should just fly to Istanbul instead.

Those questions have Lagos-specific answers, and nobody has filmed them.

So the eight hours buy answers, not awareness. Every piece should be something a coordinator can send to one named person who has already enquired, with a sentence like "she explains exactly this".

The six things that actually block a booking

Shoot against these, in this order of weight. Everything in the shot list maps to one of them.

The blocker	What it sounds like	What answers it
Who is actually treating me	"Is it her, or somebody I have not met?"	Named clinicians, how they are trained, what she signs off
Is it safe here	"I have read the stories"	What a proper consultation screens for, what gets refused, what the escalation route is
Will I look done	"I do not want to look like someone else"	Her whole position, applied to Nigerian faces and Nigerian expectations
What happens afterwards	"Who looks after me?"	The aftercare answer. The one piece of ground nobody in Lagos has taken
Why not just fly	"Turkey does the whole thing in one package"	The honest comparison, including what a flight home does not come with
What is it actually like	"What happens when I walk in?"	The consultation, demystified, step by step

What cannot be filmed, and why

Read this before the call sheet is written. Three of these have teeth.

No patients, no patient images, no before and after. MDCN's guidance of 14 October 2025 requires explicit written consent for any identifiable patient image. Her existing American before-and-afters were taken under US consents that will not cover Nigerian marketing, so they do not travel. The live precedent is a Beverly Hills plastic surgeon who lost a six hundred thousand dollar judgment, upheld on appeal in 2023, for posting patient photographs without consent.

No brand names. Botulinum toxin and dermal fillers are prescription-only medicines in Nigeria and may be advertised only in medical journals. Say "botulinum toxin" and "dermal filler". Never the trade names. Her existing library names one of them in a title, which is exactly why that video cannot be used as Nigerian paid media.



No treatment performed on camera, on anybody, until her registration is settled. Filming her treating a person in Lagos creates an evidentiary record of practice. Hands-only b-roll with a colleague, or equipment alone, until the MDCN position is in writing.

She educates, the clinic books. Self-advertisement by a practitioner is a disciplinary offence under Part F. Every call to action belongs to MedLYFE, not to her. She can say what she thinks; she cannot say come to me.

No promises. No guaranteed result, no cup size, no date back at work, no claim anything is risk free. These are the exact phrases that turn education into an advertisement.

One more, which is commercial rather than legal: **do not say prices on camera.** They are still moving, and a figure said out loud in October is wrong by January and lives on the internet forever.

Three sites, five settings

The settings are chosen so that a viewer can tell, within two seconds and with the sound off, which kind of piece they are watching.

Setting	Site	The look	What it carries
1. The consultation room	The clinic	Her seated, as if across from a patient. Daylight, real room, no styling	The highest intent content. What happens at a consultation, the screening, the questions she asks
2. The treatment room	The clinic	Standing, clinical, equipment visible. Hands-only b-roll	Procedure explainers and the safety architecture. No patient in frame, ever
3. The dark portrait set	Studio	The look from her existing portraits. Near-black ground, single source, gold mark	The authority pieces. What she will not do, why she refuses things, the position statements
4. The warm interior	House or members club	Soft daylight, seated low, conversational, no desk	The long-form conversations and the pieces that need to feel like talk rather than teaching
5. The Lagos exterior	Same site as four, terrace or entrance	Identifiably Lagos without being a postcard. Golden hour	The "I am here" pieces, the arrival b-roll, and the cutdowns that make the library feel local

Settings one and two are the same site, four and five are the same site. That is three moves across the day, which is what makes eight hours realistic.

What eight hours actually yields

Be honest about this with whoever is booking the crew. The number is set by how many scripts are signed off beforehand, not by how long the camera runs.

	Count	Length
Long-form conversations	6	Four to eight minutes



	Count	Length
Short pieces	40	Thirty to ninety seconds
B-roll	Enough to cut all of the above	

That is roughly fifty publishable assets. At three a week it is four months of publishing, which takes the practice through the evening on the twenty-first, through December when the diaspora arrives, and into January.

It only works if every script is written and approved before the day. A shoot where the script is being argued on set produces about a third of this. Scripts are not the crew's job and they are not hers; somebody has to own them a week out.

The shot list

Setting one: the consultation room, around 2 hours

The most valuable hour of the day. This is what a coordinator sends to someone who has enquired and gone quiet.

- **What actually happens at a consultation** (long form). Walk the whole thing, from the intake form to the written plan.
- The questions she asks before she will discuss any procedure
- Why the intake form has to be done before, not in the room
- What she is looking at while you are talking
- Why she asks about nicotine, and what the answer changes
- Why she asks whether your weight is still moving
- What a written plan contains, and what it deliberately does not
- "Tell me what you want to change" and why she starts there rather than with a menu
- What happens if she thinks you should not have the procedure
- What happens if you change your mind afterwards
- Bringing someone with you, and whether it helps
- What to do if you have a question after you leave

Setting two: the treatment room, around 1 hour 30

Generic names only. Hands and equipment, no faces, no patients.

- **The safety architecture** (long form). Who is in the room, what is on the trolley, what happens if something goes wrong
- What is actually in a consent conversation
- Botulinum toxin: what it does, who it suits, who it does not
- Dermal filler: the same, plus what she will not fill
- Bio-remodelling and skin boosters, honestly, including where the evidence stops
- Regenerative treatments: what is supported and what is being sold ahead of the evidence
- What "complication management" means in practice, and why a clinic should be able to describe it
- Why she will not treat on the same day as the consultation

Setting three: the dark portrait set, around 1 hour 30



Her position pieces. Shortest to shoot because they are closest to what she already says.

- **Why I will not do certain things** (long form). Her refusal list, and why the list matters more than the menu
- Looking rested is not the same as looking younger
- The work nobody can point at
- Why ageing is not the problem people think it is
- What "natural" actually means, and why everyone claims it
- Darker skin is not lighter skin with more pigment
- Keloids and scarring: what changes the plan
- The most overdone procedure right now, and why
- Why she does not want you to need her twice a year
- What it means that she trains the people who treat you

Setting four: the warm interior, around 2 hours

The conversational register. Two of these should be with a second person on camera: her and the clinic's lead clinician, which answers the "who is actually treating me" blocker better than she can alone.

- **Who treats you when she is not in the country** (long form, two-hander with the lead clinician)
- **Aftercare: the part nobody quotes for** (long form). The strongest single piece in this shoot
- Who is responsible for your recovery, by name
- What happens at two in the morning
- What is in the quote and what arrives as a separate bill
- Why recovery costs more than people plan for
- What a realistic first fortnight looks like
- When to call somebody and when not to worry
- Bringing a mother, a sister or a friend into the recovery plan
- What she wishes patients asked before, rather than after

Setting five: the Lagos exterior, around 1 hour

Short, atmospheric, and the pieces that make the library unmistakably local.

- **Why do this in Lagos rather than fly** (long form). The honest comparison, including what Turkey genuinely does well
- What a flight home does not come with
- Why she is building here
- What she was told would not be possible in Lagos
- Arrival and walking b-roll, for cutdowns and for the invitation film
- Silent portrait set, stills and motion, for the page and the invitations

The day

08:00 Crew in at the clinic. Light setting one

08:45 **Setting one, the consultation room**

Long form first, while she is freshest



10:45	Relight to setting two	Hair and makeup check
11:00	Setting two, the treatment room	Hands and equipment b-roll last, so she can leave the room
12:30	Travel and lunch	
13:30	Setting three, the dark portrait set	Her own look, so least setup
15:00	Relight to setting four	Second clinician arrives for the two-handers
15:15	Setting four, the warm interior	Aftercare long form is the priority piece. Shoot it first in this block
17:15	Setting five, the exterior	Golden hour. Hard stop when the light goes
18:15	Wrap	

The two pieces that must not be lost if the day overruns are **what actually happens at a consultation** and **aftercare**. Shoot both early in their blocks. Everything else can be picked up.

What has to exist before the day

	Who
Every script written and signed off, with the compliance rules applied before she sees them	To assign, one week out
A teleprompter, and her rehearsed on the long-form pieces	Crew and her
The second clinician confirmed for the two-handers, and briefed	The clinic
Written confirmation that nothing filmed shows her treating a patient	The clinic
Three wardrobe changes, one per site, agreed against the settings	Her
A named owner for the publishing calendar, so the library does not sit on a drive	To assign

After the shoot

The library is worth nothing on a hard drive. Three rules make it work.

Publish on a calendar, not on inspiration. Three a week, scheduled, for four months. The research is unambiguous that consistency beats volume.

The coordinator sends these one to one. That is the whole point of bottom of funnel content. Somebody enquires, goes quiet, and gets a WhatsApp message with one video that answers the specific thing they hesitated over. That is worth more than any number of public views.

Cut each long form into its own shorts. Six long pieces will yield another twenty or thirty cutdowns, which roughly doubles the library for the price of an editor rather than a second shoot day.